SAMPLE ANSWER · SPINAL CORD INJURY · CASE 4

Sample 7-minute ISBAR: Chloe

Chloe Bennett · 26-year-old woman · Inpatient spinal rehabilitation unit, public hospital · about 1037 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

54 words

"Hi, I'm the occupational therapy student who has been completing Chloe Bennett's dressing and showering session. Chloe is 26 and has T4 AIS B paraplegia from a car crash seven weeks ago. I need your help urgently because I'm seriously concerned about Chloe's safety. I think she may be at immediate risk of suicide."

S Situation

126 words

"During our session, Chloe told me she has been hiding her night-time tablets in her toiletry bag and pretending to swallow them. She said, 'I've got enough now'. She asked me not to tell anyone and said I'm the only person she has told. When I asked what she meant, she said, 'Forget I said anything'.

"I also found a note in her communication book from last Sunday recording that Chloe told Josh, her partner, she'd 'rather be dead than live like this'. I can't find any record that this was followed up.

"Chloe now says she is fine and wants to be left alone. The toiletry bag is on her lap. I haven't left her, and I'm asking for help before I do anything else."

B Background

172 words

"Chloe has been on the spinal rehab unit for three weeks. She is a NIISQ participant. Before her injury she worked as a teacher's aide and played netball. She lives with Josh in a two-storey townhouse in Brisbane.

"She is medically stable and physio reports good progress with sitting balance and wheelchair skills. Nursing describe her as 'quiet but settling in'. However, she has missed four therapy sessions in the past week, mostly recorded as 'declined – tired'.

"Today she was in bed with the blinds closed, her breakfast untouched and unread messages from her netball team. She dressed with minimal assistance but barely spoke and avoided eye contact. When the recreation officer came in, she smiled brightly.

"There are important gaps. I can't find any mental health or risk assessment since admission, and I don't know if she's seen psychology or social work. I don't know how many tablets she has, what they are, or whether she has any other means or plans. I also don't know her mental health history."

A Assessment

255 words

"I think this needs to be treated as an immediate risk. Chloe has expressed a wish to be dead and has made a specific preparation, collecting medication and saying she has 'enough now'. She has asked me to keep it secret. These are serious warning signs of possible suicide risk.

The previous statement to Josh shows this isn't a one-off comment, and it doesn't appear to have been acted on.

"Around this, there's a cluster of signs of low mood and withdrawal: missed sessions, staying in bed, not eating, avoiding her friends and limited engagement. Depression and adjustment difficulties are common after spinal cord injury, especially in the early months when people are facing major losses in roles, independence and identity. For Chloe, that includes her work, netball and her relationship.

"Her 'I'm fine, just tired' and her bright smile for the recreation officer could easily reassure people, but I don't think they reduce the risk. Medical stability and good physio progress tell us about her body, not her mood. The NIISQ forms, her mum's house photos, Josh's questions about sexuality and fertility, and the sports day are all important parts of her rehab, but none of them can be the focus today.

"I can't keep this confidential. Her safety has to come first, and confidentiality has limits when someone may be at serious risk. As a student, I'm not the right person to complete a formal suicide risk assessment or decide on treatment. But this needs urgent escalation and nobody should leave her alone."

R Recommendation

430 words

"My immediate priority is to stay with Chloe. I'd use the call bell or ask a staff member to get the nurse in charge and to let you know, and I'd ask someone to tell my next client I'm delayed. I won't leave her alone with the toiletry bag.

"I'd be honest and kind with her. I'd say I care about what she's told me, that I can't keep it secret because I'm worried about her safety, and that people here want to help. I'd ask her directly, in a calm way, whether she is thinking about ending her life, because asking directly doesn't increase risk and shows I'm taking her seriously. I'd listen without judgement.

"Second, I'd like your help to escalate this to the nurse in charge and the unit's medical officer straight away. Nursing need to secure the tablets respectfully and with her as involved as possible, check her room for other medications or means, and review how night medications are given. I'd request an urgent medical and mental health review today, which could involve the psychiatry or mental health liaison team. They would assess her risk and decide on monitoring and treatment. I'd also flag that last Sunday's note wasn't followed up so this can be reviewed.

"Third, the visitors and tasks can wait. Nursing or social work can let her mum know there's a delay without sharing details. I'd ask the team to check with Chloe about what she wants Josh and her mum to know and how she'd like them involved in her safety plan. The NIISQ forms and the sports day can wait until she's been reviewed.

"Fourth, once she's safe, I'd ask for psychology and social work input, and later a referral to the unit's sexuality and fertility service for Josh and Chloe's questions. Peer support through Spinal Life Australia may help too, if Chloe wants it.

"For OT, I wouldn't push the shower today. Once the mental health team supports it, my principal short-term assessment would be observed performance of a self-care routine Chloe chooses. This would help rebuild a sense of control and show us where she needs support.

"Longer term, I'd use the COPM, because her goals have to come from her: work, netball, her relationship and her home. OT can then help her rebuild meaningful routines and roles, which

supports both rehab and wellbeing.

"The NSQHS Comprehensive Care Standard requires services to recognise and respond to self-harm and suicide risk. My priority right now is staying with Chloe, securing the medication and getting urgent mental health review today."

Coaching notes *not spoken*

The immediate priority

Chloe has disclosed a stockpile of medication, said she has "enough now", and has previously said she'd rather be dead. This is an immediate suicide risk. Stay with her, do not keep the secret, have nursing secure the medication, and arrange urgent medical and mental health review.

Distractors in this case

- **"Fine, just tired" and a bright smile** — masking is common; her behaviour tells a different story.
- **Medically stable, good physio progress** — this tells you nothing about her mood or risk.
- **NIISQ forms, mum's house photos, the sports day** — important later, but not today.
- **Josh's sexuality and fertility questions** — a valid referral once Chloe is safe.
- **Your next client waiting** — get someone else to tell them; don't leave her alone.
- **"Please don't tell anyone"** — confidentiality does not apply when someone may be at serious risk.

Missing information to clarify

- Number and type of tablets; any other means
- Any formal risk assessment, psychology or social work input
- Mental health history
- Why the earlier note wasn't acted on
- Who Chloe wants involved

Useful references

- Australian Commission on Safety and Quality in Health Care, *NSQHS Standards – Comprehensive Care Standard* (minimising harm: self-harm and suicide)

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